

OUTLINE OF EVIDENCE – MR CORY LEA SHEPHERD (APPELLANT)

Served pursuant to direction 2 of the Further Directions Order (3) dated 19 August 2026. Not filed in the Industrial Registry.

I will give evidence of the following. Paragraph references are to the notice to admit facts served 28 August 2026 (the Notice); its Annexure A holds the documents referred to.

- 1. Employment and the role.** I commenced at Logan Hospital Switchboard on 25 March 2019 as a casual, became permanent from 3 March 2021 and full time from 16 October 2023, as a continuous shift worker. The duties: the emergency response notifications, "strictly adhering to protocols and timeframes"; the pager and on-call records on which emergency paging depends; and some 200 to 300 calls a shift, including urgent clinical handover and distressed patients and families; and why an accurate entry matters where an urgent page goes to whatever number is recorded. My background: safety advisor on Manus Island, Papua New Guinea, and earlier a third-year Bachelor of Nursing student. [Notice ¶¶ 1–16, 26–38]
- 2. Sleep and fatigue.** My sleep on rotating shifts across 2023 and 2024 and the melatonin and temazepam prescribed on 16 November 2023. The shifts of 17 and 18 March 2024: my travel time each way, finishing at 23:00 and starting at 06:00, and the rest and sleep actually available; that on the evening of 17 March I handled a MET call and, four minutes later, a Code Blue to the same bed, then a further MET call; that 18 March was a Monday day shift, which carries the week's peak call volume and on which the queue is rarely cleared, and that between 06:00 and 14:00 I handled a Code Grey, three MET calls and two neonatal MET calls while answering that queue of urgent handover, distressed callers and complaints; and the sick leave of 19 March; that those shifts were not a staff-initiated swap, the only case in which, by Ms Forrest's email of 7 July 2026, the 8-hour agreement applied. That when I raised fatigue, neither my manager nor the Director applied any fatigue protocol to my roster: no fatigue risk assessment was made, no FRMS register was kept, and the toolkit at clause 10.4.1 of the Operations Manual, which I sent to Ms Taylor, had no response; my request of 8 April 2024 and the refusal of 1 May 2024; and that Metro South has since stated in writing that the fatigue risk assessment records and register entries requested "do not exist", that FRMS training applied only to clinical staff, and that fatigue risk management assessment at the Switchboard began only after 30 June 2024. [¶¶ 17–25, 211–256, 263–268]
- 3. Rostering: lates then earlies.** The pattern on my roster line of late shifts followed by early shifts, and of nights, days off, then nights again, raised on 16 April 2024; that rostering errors on my line recurred across successive roster periods; and Ms Reese's acknowledgement of 10 May 2024 of "a few rostering errors made by Chloe with regards to Cory's line in past rosters". [¶¶ 211–223]
- 4. Pay and entitlements.** The public holidays I was not rostered between February and April 2024 and the difference to my pay. The loop: on 8 April 2024 I asked Ms Taylor to review my pay; on 9 April she directed me to raise the older dates through MyHR payroll enquiries; Payroll told me the payments had to be processed by my manager, which I relayed to her on 24 April; on 3 May Payroll wrote to her, copying me, to "submit an AVAC to correct these shifts for each fortnight"; when I followed up with Payroll on 10 May, Payroll on 13 May told me to "speak to your Line Manager"; on 21 May she was still "waiting payroll confirmation"; on 28 May I was asked to sign a validation of claims older than three months. That under the payroll process only my manager could submit the AVAC. The February 2024 special pandemic leave application: what I attached, the two declines, and the approval on the same material. [¶¶ 114–142, 182–210, 242–246]
- 5. The database, the misdirected calls, and what I could not fix.** That from 18 July 2023 operators could no longer correct database entries and the Contact & Number Changes book was removed, so a wrong number waited on Ms Stibbard or Ms Taylor and after hours had to "wait until either Chloe or myself are back"; the directives of 15 April, 19 April and 9 May 2024, and that I was not consulted before any of them. That from 15 April 2024, when Ms Taylor notified the department that she and Ms Stibbard had placed themselves on after-hours call, departments and clinicians ringing during my shifts with pager and phone updates asked for her, and on many occasions neither they nor I could have the update or issue resolved during the shift. The MASPER Registrar's "Switchboard issues" emails to Ms Taylor of 3 May 2024 at 3:06 pm and 8 May at 5:28 pm, nine calls reaching the wrong team, including the MET call team ringing because "switchboard could not tell them where VHUB was"; that her first response came on 9 May at 9:20 am, five days and eighteen hours after the first, asking Dr Wong for her business hours because they were "not provided on the rosters" and noting her DECT phone "is currently switched off", the manager herself describing the difficulty I describe in this paragraph and the next; and at 10:15 am telling the team of "many ongoing issues" about calls "transferred to the wrong medical teams"; the Integrated Respiratory Service writing on 15 and 20 May 2024 that "we can not help patients or other clinical staff"; and that on each occasion I could not correct the entry myself and the delay built up while the correction waited. [¶¶ 39–113]
- 6. Ms Taylor's hours: the question, the retraction request, and the answer.** That neither I nor the department knew her office hours; my email of 15 May 2024 at 1:15 pm asking her to share them and that changes be made in consultation with the team; Ms Reese's reply that evening asking me to retract it, which I did; that two days later, on 17 May 2024 at 9:30 am, Ms Taylor sent the whole department an email stating "Otherwise my hours are from 06:30-14:30", the very information I had asked for; and Ms Reese's reply of 21 May 2024 asking me to identify the directives I was concerned about. [¶¶ 70–88]
- 7. The book, the earlier concerns, and the delegate role.** The communication book entry of 6 June 2023 and its removal; the concerns I raised on 7 August 2023 and the responses; my complaint of 13 May 2024, later determined to be a public interest disclosure (content not set out); and my interest in becoming the Switchboard union delegate and what followed. [¶¶ 143–181, 283–299]
- 8. My health before June 2024.** That before 18 June 2024 I had never been diagnosed with or treated for depression. My general practitioner's record of 16 November 2023 notes poor sleep with shift work, that I could not do shifts without a good sleep, no psychological illness such as depression or psychosis, and mood good. The ADHD and anxiety noted in my history in October 2022, and the referral to Dr Amini renewed on 16 May 2024 for which no appointment could be obtained. [General-practice records disclosed by the Respondent]
- 9. Onset, presentation and diagnosis.** My symptoms from about 18 June 2024 and their effect on sleep, concentration, self-care and relationships, and on my capacity for work, from being certified unfit from 1 July 2024 to returning on reduced hours and the restrictions recorded in the Employee Capability Checklist of 3 July 2026. Attending Dr Slawinski on 28 June 2024, whose note records "stress at work" and "upset by people not following rules", and Dr Hawes on 1 July 2024, whose note records "work stress", that "they withhold pay at times, no overtime- not processed, manipulate his roster- so he works lates then earlies". Lodging my application for compensation on 1 July 2024. Dr Hawes's certificates of 1 July and 8 September 2024 and his psychiatrist referral. My first consultation with Dr Krishnaiah on 24 October 2024, on that referral, when I was told I had major depressive disorder with anxiety state, confirmed by the practice in writing at 11:45 am that day, my fluoxetine increased and quetiapine added at night; his written report of 13 February 2025; and my treatment since. The matters at paragraphs 2 to 7 above are the matters I reported to each doctor, as their records show. I offer no clinical or diagnostic opinion.
- 10. After 18 June 2024, and documents.** Meetings scheduled while I was on certified leave; my employment ended under abandonment provisions in October 2024 while I held current certificates, and my later reinstatement; my return on reduced hours and the leave since; that my Queensland Health email was restricted, so I cannot produce my work emails myself. Documents: rosters and payslips; my emails and messages with my line manager and the Director; the employer's leave and payroll records; the role description; the medical records above; and the Metro South Hospital and Health Service letter of 5 June 2026 (ref K-LM26/729).

This outline states the topics on which I will give oral evidence. It is not a statement of evidence and is not verified.